MN Health Care Coordination for Individuals with Physical Disabilities

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TRiO RONALD E. McNAIR POST-BACCALAUREATE ACHIEVEMENT PROGRAM

STATEMENT OF THE PROBLEM

- Individuals with physical disabilities (PD) encounter many challenges and barriers to obtaining well-coordinated, comprehensive, and accessible healthcare.
- A significant challenge to obtaining quality care is the current lack of coordination in services and supports provided to individuals with PD and other complex health care needs.
- Although crucial to implementing an effective Healthcare Coordination (HCC) program, there is a current lack of information as to which aspects of HCC are most critical to patient outcomes.
- No existing conceptual framework for effective HCC has included input from the patients with PD and rarely have Care Coordinator's (CC) themselves been asked to provide input.
- It is unlikely an effective model of HCC and the role of CCs can be developed without input from multiple stakeholder groups, including: healthcare administrators/supervisors, CC, patients with PD, and guardians of individuals with PD.

BACKGROUND

- Existing HCC programs employ a variety of models, each of which possess a set of features assumed to be critical to their success.
- In almost all instances, however, there is little to no empirical data to support the inclusion of one set of "critical" elements over that of others.
- In addition, a model of HCC specifically designed to meet the needs of individuals with physical disabilities has yet to be developed and empirically validated.

PURPOSE OF RESEARCH

- To create a HCC model containing elements that are empirically linked to increased access and quality of health care, decreased costs and utilization, and improved consumer health outcomes for individuals with PD.
- The goal of this phase of the multiphase project is the identification of those elements of HCC deemed critical for effective care coordination as identified by patients with PD.

METHODOLOGY

Requested stakeholders with PD to identify critical characteristics of effective Care Coordinator (CC) in Hennepin, Ramsey, and surrounding counties via initial telephone interview.

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Responses quantitatively analyzed and categorized into four Domains by project staff

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NVivo program (qualitative analysis) assisted with creation of preliminary HCC models that included elements of HCC deemed critical to its success by consumer stakeholders .

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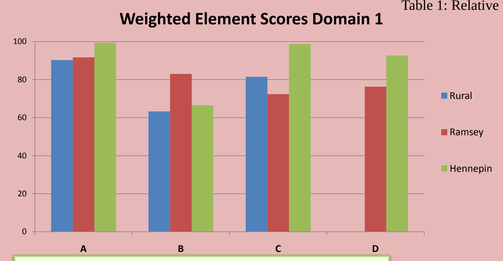
PPDM groups and project staff analyzed preliminary HCC models and organized elements into a hierarchy based on relative importance assigning weights to each Domain and Element ranging from 00-100.

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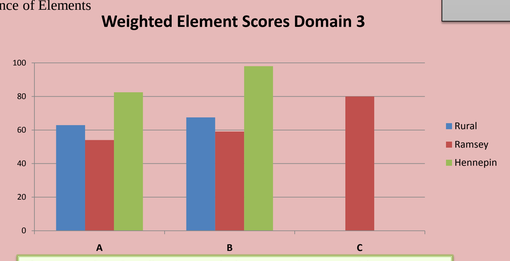
Percentage weights were calculated by averaging PPDM rankings for each element and multiplying by the corresponding mean domain weight.

RESULTS

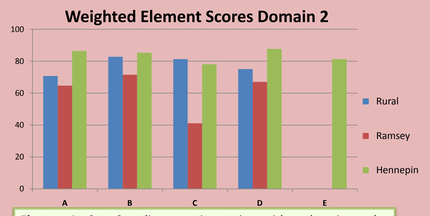
Table 1: Relative Importance of Elements



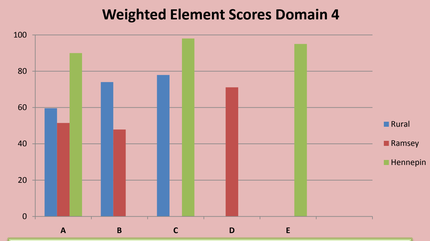
Element A: Care Coordinator interviews patient in person
Element B: Care Coordinator interviews patient by phone
Element C: Care Coordinator inquires about patient's doctor, medication, healthcare needs and concerns
Element D: Care Coordinator determines patient eligibility for services based on knowledge of providing MCO



Element A: Care Coordinator attends doctor appointments with patient
Element B: Care Coordinator speaks with and receives information from doctors on patients behalf
Element C: Care Coordinator has unrestricted access to patient records within the provider agency

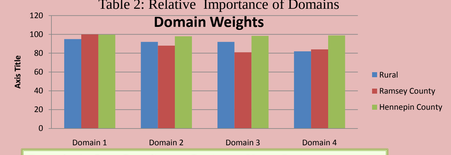


Element A: Care Coordinator assists patient with goal setting and accomplishment
Element B: Care Coordinator checks in with patient periodically in-person, by phone, or via email
Element C: Care Coordinator provides education on health, wellness, and chronic illnesses
Element D: Care Coordinator maintains contact with patient during and after hospitalizations and nursing home stays
Element E: Care Coordinator offers to attend doctor appointments with patient



Element A: Care Coordinator links patient to transportation, medical equipment, and physical therapy
Element B: Care Coordinator links patient to assistive technology supports
Element C: Care Coordinator links patient to Home Health services.
Element D: Care Coordinator assists patient with resolving insurance issues
Element E: Care Coordinator links patient with social and employment services

Table 2: Relative Importance of Domains



Domain 1: Care Coordinator Getting to Know Patient
Domain 2: Care Planning and On-going Monitoring
Domain 3: Care Coordinator Communication with Doctor
Domain 4: Patient Access to Information and Services

DISCUSSION

Table 3: Preliminary HCC Model

Care Coordinator Getting to Know you	Care Planning and On-Going Monitoring	Care Coordinator Communication with Doctor	Access to Information and Services
• Interview patient in person to initiate rapport • Interview patient by phone • Inquire about doctors, medications, healthcare needs • Determine patient eligibility for services at initial assessment based on knowledge of providing MCO	• Assist with goal setting and accomplishment • Check in periodically by phone, email, or in-person (if it has been a while) • Provide education on health, wellness, and chronic illness • Maintain contact during and after hospital and nursing home admissions • Offer to attend doctors appointments	• Attend doctor appointments at patient request • Speak with doctor and receive information on patients behalf • Unrestricted access to patient records within the provider agency • Adjust intensity of services to accommodate patients desired level of autonomy	• Link to transportation services, medical equipment and therapy (e.g. physical therapy) • Link to assistive technology supports • Link to Home Health services • Assist patient with resolution of insurance issues. • Link with social and employment services.

Implications

- Addition to current literature: Identification of critical elements of HCC not previously recorded: assistance resolving insurance issues, links to health clubs, social, and employment services, education regarding health and wellness and specific disabilities
- PPDM results will be combined with 6 additional PPDM groups conducted with adults with PD to develop the first HCC model based on critical elements as defined by consumers with PD

Limitations:

- Convenience sampling from HCC programs
- Small sample size (n=18)

Conclusion

The culmination of this phase of project will result in the first HCC model that includes input form 4 stakeholder groups: healthcare administrators/supervisors, Care Coordinators, consumers with PD, and guardians of consumers with PD. The HCC model will contain critical elements that are empirically linked to increased access and quality of care, decreased cost and utilization, and improved consumer outcomes for consumers with PD.

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